

SPIRAL CT SCAN OF WITH 3D RECONSTRUCTION, WITHOUT CONTRAST:

No sign of soft tissue swelling or edema is seen.

No sign of lytic or sclerotic lesion is seen.

No sign of fracture line is seen.

The bony part is unremarkable

SPIRAL CT SCAN OF ABDOMEN AND PELVIC WITHOUT CONTRAST + coronal, reconstruction:

Multiple axial, coronal reconstruction images through the abdomen and pelvic without contrast administration demonstrate:

The liver is normal in size with normal parenchymal density with no evidence of mass or biliary dilatation.

The G.B is unremarkable.

Pancreas has normal size and density with no sign of S.O.L.

Spleen has normal size and density with no sign of S.O.L.

Both adrenal glands are normal.

Both kidneys are normal in size and density with no sign of stone, stasis or abnormal density.

No hydronephrosis or ureteral dilatation is seen.

There is no sign of peripancreatic or para aortic lymphadenopathy.

There is no sign of mesenteric stranding.

The hollow viscous structures have normal distribution.

Urinary bladder is normal with no sign of stone or wall thickening.

Other genito-urinary structures in pelvis show normal density and anatomic configuration.

There is no sign of pelvic side wall lymphadenopathy.

The visualized part of the bony skeleton are normal in density and shape.

No free fluid is noted in abdomino pelvic cavity.

SPIRAL CT SCAN OF ABDOMEN AND PELVIC WITH CONTRAST + coronal, reconstruction:

Multiple axial, coronal reconstruction images through the abdomen and pelvic after contrast administration demonstrate:

The liver is normal in size with normal parenchymal density with no evidence of mass or biliary dilatation.

The G.B is unremarkable.

Pancreas has normal size and density with no sign of S.O.L.

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No free fluid is noted in abdomino pelvic cavity.

SPIRAL CT ANGIOGRAPHY OF THORACIC AORTA AND CTA OF ABDOMINAL AORTA AND CTA OF BOTH LOWER EXTREMITIES (PELVIS LEG FEMUR) AND ILIAC ARTERIES+3D IMAGES WITH IV CONTRAST:

Multiple axial images taken through thoracic aorta and abdominal aorta and both lower extremities cavity after injection of IV contrast and 3D reconstruction revealed:

M.R.I OF THE ANKLE & SUBTALAR JOINTS without contrast:

The bones comprising the ankle joint show normal configuration, with normal development of the ankle mortise.

The bone marrow signal, trabecular pattern, and epiphysial lines are all normal.

The joint space is of normal width.

The cortex shows normal thickness and smooth contours, especially along the tibial and talar articular surfaces.

There are no subchondral signal changes and so osteophyte.

The lateral and medial ligaments are normal in their course, width, and signal characteristic.

The talocalcaneal and talonavicular joints appear normal.

The intraosseousligaments between the talus and calcaneous is intact.

The

n were taken which demonstrate:

The cervical spine shows a normal degree of lordosis.

All visualized vertebral bodies show normal height and signal intensity, without any sign of mass or fracture or compression.

The intervertebral disc spaces are preserved. Also no sign of significant disc bulging or protrusion is seen.

The intervertebral neural foramina appear normal.

Visualized parts of cord, including cervicomedullary junction appear normal in position, size and signal intensity, without any sign of edema or hemorrhage or mass or atrophic changes.

No extra or intra-dural mass is seen.

Posterior elements appear normal.

SPIRAL CT SCAN OF CHEST AND MEDIASTINUM : (WITHOUT CONTRAST) + coronal reconstruction

Multiple axial, coronal reconstruction images through the chest and mediastinum demonstrate:

Normal lung markings.

No active pulmonary infiltration seen.

There is no sign of interstitial lung disease.

The bronchial system are visualized to have normal caliber with no sign of peribronchial infiltration.

No abnormal dilatation of bronchial system or bronchiectasis is noted.

The heart and mediastinum are normal with no sign of lymphadenopathy.

Bony thorax and soft tissue structures are apparently normal in anatomic configuration and density.

No sign of pleural effusion is seen.

SPIRAL CT SCAN OF CHEST AND MEDIASTINUM : (WITH CONTRAST) + coronal reconstruction

Multiple axial, coronal reconstruction images through the chest and mediastinum demonstrate:

Normal lung markings.

No active pulmonary infiltration seen.

There is no sign of interstitial lung disease.

The bronchial system are visualized to have normal caliber with no sign of peribronchial infiltration.

No abnormal dilatation of bronchial system or bronchiectasis is noted.

Heart size is normal.

No mediastinal or hilar lymphadenopathy is seen.

Bony thorax and soft tissue structures are apparently normal in anatomic configuration and density.

No sign of pleural effusion is seen.

T-TUBE CHOLANGIOGRAPHY :

After injection of contrast agent into T-TUBE normal filling of biliary tree is seen .

Normal passage of contrast from biliary tree to bowel loops is seen without any obstruction or filling defect or leakage .

CONCLUSION : Normal T-tube cholangiography.

MRI of CP Angles with & without Contrast:

Multiplanar multisequential MR images through CP angles with & without contrast were taken, which demonstrate:

Both CP angles have normal configuration.

The course of 7th-8th cranial nerve is normal.

No vascular abnormality is seen.

Posterior fossa including cerebellum and pons are normal.

No sign of mass lesion is seen.

No abnormal enhancement is noted after injection of contrast.

SPIRAL CT SCAN CERVICAL SPINE WITHOUT CONTRAST WITH SAGITTAL CORONAL 3D RECONSTRUCTION :

The visualized vertebrae have normal height and alignment .

The vertebral bodies have normal shape with no evidence of fracture.

The posterior elements also are normal with no evidence of fracture line or dislocation.

No lytic or blastic bony lesion is noted.

SPIRAL CT SCAN OF ABDOMEN AND PELVIC + CT ENTROGRAPHY WITH CONTRAST + coronal, reconstruction:

Multiple axial , coronal reconstruction images through the abdomen and pelvic after iv and oral contrast administration demonstrate:

Small Bowel: The jejunum and ileum are adequately distended. Normal wall thickness and enhancement are noted throughout.

No mural stratification, hyperenhancement, ulceration, skip lesions, or strictures are seen. No evidence of fistula, sinus tract,

or abscess formation.

Colon: Visualized portions of the colon demonstrate normal wall thickness and enhancement, without inflammatory changes.

Mesentery: No mesenteric fat stranding, comb sign, or enlarged mesenteric lymph nodes.

Stomach and Duodenum: Unremarkable.

The liver is normal in size with normal parenchymal density with no evidence of mass or biliary dilatation.

The G.B is unremarkable.

Pancreas has normal size and density with no sign of S.O.L.

Spleen has normal size and density with no sign of S.O.L.

Both adrenal glands are normal.

Both kidneys are normal in size and density with no sign of stone, stasis or abnormal density.

No hydronephrosis or ureteral dilatation is seen.

There is no sign of peripancreatic or para aortic lymphadenopathy.

Urinary bladder is normal with no sign of stone or wall thickening.

Other genito-urinary structures in pelvis show normal density and anatomic configuration.

There is no sign of pelvic side wall lymphadenopathy.

The visualized part of the bony skeleton are normal in density and shape.

No free fluid is noted in abdominopelvic cavity.

SPIRAL CT ANGIOGRAPHY OF abdominal aorta+ 3D IMAGES WITH IV CONTRAST:

Multiple axial images taken through abdominal aorta after injection of IV contrast and 3D reconstruction revealed:

All abdominal vessels including celiac trunk and its major branches, superior mesenteric artery inferior mesenteric artery , and both renal arteries are patent with normal internal opacification and no evidence of abnormal narrowing or dilatation.

Abdominal aorta is normal in diameter shape and internal opacification with no evidence of aneurysmal dilatation.

No abnormal narrowing or calcified plaque noted in the course of aorta.

SPIRAL CT ANGIOGRAPHY OF BRAIN(CORONAL, AXIAL , SAGITTAL)MIP+3D WITH CONTRAST:

Multiple axial images through the brain (ct angio) injection which reveals:

Both sides ACA, MCA, PCA, vertebrobasilar arteries, ACOM and PCOM show normal course with no evidence of narrowing or aneurysmal dilatation.

No sign of filling defect is seen in mentioned vessels.

No sign of AVM is seen.

SPIRAL CTA OF BOTH LOWER EXTREMITIES (PELVIS, LEG, FEMUR) and iliac arteries + 3D RECONSTRUCTION WITH CONTRAST :

Multiple coronal images through the both lower extremities CT angio demonstrate:

Both sides lower extremity arteries including common femoral, superficial femoral and deep femoral, popliteal and distal arteries including dorsalis pedis and posterior tibialis artery are normal in diameter and are filled adequately with contrast agent without narrowing or stenosis.

No evidence of arterial wall injury in total course of both lower extremity arteries is noted.

SPIRAL CTA OF NECK (SAGITAL CORONAL 3D RECONSTRUCTION) WITH IV CONTRAST:

Multiple axial images through the neck (ct angiography) with contrast demonstrate:

Both sides CCA, ECA, ICA and vertebral arteries show normal caliber and course with smooth intima and with no narrowing and obliteration .

Carotid arteries bifurcations in both sides show normal configuration with no filling defect or plaque or narrowing.

No abnormal dilatation is seen in all mentioned vascular structures too.

SPIRAL CT ANGIOGRAPHY OF THORACIC AORTA +3D WITH IV CONTRAST :

Multiple axial images taken through thoracic aorta after injection of IV contrast and 3D reconstruction revealed:

Ascending and descending aorta shows normal configuration and normal caliber with no sign of thrombosis or stenosis.

No sign of aneurysmal dilatation is seen.

No sign

of dissection is detected.

SPIRAL CTA OF BOTH UPPER EXTREMITIES (ARM, FOREARM) +3D RECONSTRUCTION WITH CONTRAST:

Multiple coronal images through Both upper extremities CTA demonstrate:

Both sides upper extremity arteries including subclavian artery, axillary artery, brachial artery and antecubital artery and radial and ulnar artery are normal in diameter and are filled adequately with contrast agent without narrowing or stenosis.

No evidence of arterial wall injury in total course of both upper extremity arteries are seen.

No aneurysmal dilatation was detected.

Conclusion: Normal CTA of both upper extremities arteries is noted.

(BRAIN CT VENOGRAPHY) SPIRAL CT ANGIOGRAPHY OF BRAIN(CORONAL, AXIAL , SAGITAL) MIP+3D WITH CONTRAST:

Multiple axial images through the brain (ct venography) injection which reveals:

Superior sagittal, transverse, sigmoid, straight sinuses are patent with normal density with no filling defect in total course of them.

Internal cerebral vein and vein of gallon shows normal caliber with no evidence of clot formation within them.

DEFECOGRAPHY:

After insertion of rectal tube and injection of contrast agent study reveals:

Recto anal angle in;

Rest: degree.

Squeeze: degree.

Defecation: degree.

SPIRAL CT SCAN OF ABDOMEN AND PELVIC + DYNAMIC OF LIVER+ WITH CONTRAST + coronal, reconstruction:

Multiple axial, coronal reconstruction images through the abdomen and pelvic with contrast administration demonstrate:

The liver is normal in size with normal parenchymal density with no evidence of mass or biliary dilatation.

The G.B is unremarkable.

Pancreas has normal size and density with no sign of S.O.L.

Spleen has normal size and density with no sign of S.O.L.

Both adrenal glands are normal.

Both kidneys are normal in size and density with no sign of stone, stasis or abnormal density.

No hydronephrosis or ureteral dilatation is seen.

There is no sign of peripancreatic or para aortic lymphadenopathy.

There is no sign of mesenteric stranding.

The hollow viscous structures have normal distribution.

Urinary bladder is normal with no sign of stone or wall thickening.

Other genito-urinary structures in pelvis show normal density and anatomic configuration.

There is no sign of pelvic side wall lymphadenopathy.

The visualized part of the bony skeleton are normal in density and shape.

No free fluid is noted in abdomino pelvic cavity.

M.R.I OF THE Elbow Joint without contrast:

The elbow joint shows normal configuration with normal articulation of the bone ends.

The articular surface are smooth and confirmation especially in the radiohumerl and ulnohumeral joints, with no discontinuities.

There are no osteophytes or subchondral joint changes.

The joint space are of normal width.

The olecranon fossa is clear, and there are no intra-articular loose bodies.

The cortex of the tubular is of normal thickness.

The bone marrow signal is normal.

Imaged ligamentous structures appear normal, particularly the annular ligament.

The ulnar , radial and median nerves display a normal course and diameter.

The imaged muscles show no abnormalities.

Impression: Normal elbow JOINT M.R.I.

MR ENTROGRAPHY (MRI Abdomen & pelvic without & with contrast):

PROTOCOL: Pre contrast images were obtained with cine flouro imaging as well as fat saturated T2 and also trufi images of small bowel. Pre contrast T1 W flash images were also obtained .Post contrast images were obtained using T1 flash images as well as T2 haste and trufi images.

FINDINGS:

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ejunal folds appear normal without nodularity. No small bowel obstruction is seen. Ileal loops are also normal. No area of abnormal wall enhancement or thickening or obvious was noted. No evidence of fistula formation was noted. Comb sign was not noted. Other visualized abdominal viscera were also unremarkable.

The liver is normally positioned and has normal size and smooth borders.

Its internal structure is normal with no focal abnormalities of signal intensity.

The intrahepatic and extra hepatic bile ducts are not distended.

The porta hepatis appears normal.

The gallbladder displays a normal size, Smooth borders and homogenous contents.

The spleen is orthotopic and of normal size.

It has smooth outer contours and a homogenous internal structure. The pancreas is normal in size and position.

The head and tail of the organ have smooth, lobulated outer contours and normal internal structures.

The pancreatic duct is unobstructed.

Both kidneys are normal in size and position. The renal parenchyma shows normal width and structure.

The renal pelvis and calices are normal.

The urinary drainage tract is unobstructed.

Both adrenal gland are normal in position and size and the adrenal crura are normally developed.

The adrenal compartment is unremarkable.

Major vessels and the para-aortic region appear normal, with no evidence of lymphadenopathy.

Imaged portions of the lung and soft tissues show no abnormalities.

After injection of contrast, no abnormal enhancement is detected.

The pelvic inlet appears normal with normal configuration of the iliac wings and iliopsoas muscles.

No abnormalities are found in imaged bowel structures and there are no signs of wall thickening or mass lesions.

The distended urinary bladder appears normal and has normal wall thickening.

The seminal vesicles are of normal size. The angle between the bladder and seminal vesicles is normal on each side.

The prostate shows a normal size and configuration.

The uterus is anteverted and has normal internal structure.

The adnexa appear normal on both sides.

The vessels of the lesser pelvis are normal in their course and caliber. There is no apparent lymphadenopathy.

The femoral heads are normally shaped and articulate normally with the acetabula.

They have normal bone-marrow signal characteristics.

The soft tissues show no abnormalities.

A few foci bilateral T2 FLAIR high signal intensity is seen in both periventricular white matter suggestive of Fazekas I.

Several confluent bilateral T2 FLAIR high signal intensity foci which some of them is confluent is seen in both periventricular white matter suggestive of Fazekas II.

Multiple confluent bilateral T2 FLAIR high signal intensity foci is seen in both periventricular white matter suggestive of Fazekas III.

Prominency in extra and intra axial CSF is noted associated with age related cortical volume loss.

FOOT MRI:

Multiplanar multisequential MR images through the foot reveals:

All of the visualized bony parts have normal signal intensity.

No marrow edema is noted.

All of the visualized joint space have normal width, with normal chondral surface.

No mass is detected.

There is no bone destruction or erosion.

Soft tissues are unremarkable.

All of the visualized tendons and ligaments have normal signal intensity.

M.R.I OF THE HAND without contrast:

The bone comprising the hand present a normal configuration.

The radial joint angle is normal.

The carpal bones show normal shape and relationship to one another and to the radi

ocarpal and carpometacarpal joints.

The articular surfaces are smooth and congruent with normal cortical thickness and normal width of the joint spaces.

There are no osteophytes and no subchondral signal changes.

The bone marrow signal is normal.

The ulnar (triangular) disc exhibits normal configuration and normal signal characteristics.

The carpal tunnel is of normal width and transmits tendons that are normal in width and position.

The median and ulnar nerves appear normal.

The metacarpals and phalanges have normal margins and normal bone marrow signal intensity.

The soft tissues are normal.

Impression: Normal HAND.

M.R.I OF THE HIP JOINT without contrast:

The femoral and acetabula are of normal shape, and the femoral heads are well covered by the acetabular margins.

The joint spaces are of normal width.

The articular surfaces are smooth and congruent and show normal cortical thickness.

There are no marginal osteophyte or subchondral signal changes.

The bone marrow shows normal signal intensity, especially in the femoral head and neck.

Each femoral shaft has normal margins and contains a normal bone marrow signal.

The imaged muscles and the lesser pelvis show no abnormalities.

Impression: Normal HIP JOINTS M.R.I.

SPIRAL HRCT OF LUNG INSPIRATORY AND EXPIRATORY VIEW WITHOUT CONTRAST, AXIAL AND CORONAL:

Multiple axial and coronal images through the thorax in high resolution technique without contrast administration were taken which reveal:

There is normal lung markings.

No active pulmonary infiltration is seen.

There is no sign of interstitial lung disease.

Bronchial system are visualized to have normal caliber with no sign of peribronchial infiltration.

No abnormal dilatation of bronchial system or bronchiectasis is noted.

Due to non-contrast study evaluation of heart and mediastinum is suboptimal however they grossly appear to be normal.

Bony thorax and soft tissue structures are apparently normal in anatomic configuration and density.

No sign of pleural effusion is seen.

It must be mentioned that HRCT of lung is not ideal for evaluation of mediastinum and also possible small pulmonary nodules.

For this purpose dynamic chest CT scan is helpful.

SPIRAL CT SCAN OF KUB WITHOUT CONTRAST + coronal reconstruction

Multiple axial, coronal reconstruction images through the KUB demonstrate:

Both kidneys are normal in size and density with no sign of stone, stasis or abnormal density.

Both ureters are not dilated.

Urinary bladder is normal with no sign of stone or wall thickening.

Other genito-urinary structures in pelvis show normal density and anatomic configuration.

SPIRAL CT SCAN LUMBOSACRAL SPINE WITHOUT CONTRAST WITH SAGITTAL CORONAL 3D RECONSTRUCTION :

The visualized vertebrae have normal height and alignment .

The vertebral bodies have normal shape with no evidence of fracture.

The posterior elements also are normal with no evidence of fracture line or dislocation.

No lytic or blastic bony lesion is noted.

M.R.I. of the left Knee Joint without Contrast:

Multiplanar multisequential MR images through the left knee joint, without gadolinium injection were taken which demonstrate:

There is no sign of significant joint effusion.

ACL and PCL are normal in size and signal intensity, without any sign of definite tear.

Medial and lateral menisci appear normal in signal intensity, without any sign of significant degeneration or tear.

Medial and lateral collateral ligaments are intact without any definite tear.

The quadriceps and patellar tendon

s are normal in size and signal intensity, without any evidence of tear.

The articular cartilage is preserved with no sign of degenerative changes or cartilaginous loss.

The patellofemoral joint is preserved without any sign of dislocation.

Visualized bony structures show normal signal intensity and configuration without any sign of bone bruise, fracture or mass.

Surrounding soft tissue structures are unremarkable without mass lesion or fluid collection.

Impression: Normal Knee Joint M.R.I

Lumbosacral Spine MRI without contrast:

Multiplanar multisequential MR images through the lumbosacral spine, without gadolinium injection were taken which demonstrate:

The lumbar spine shows a smooth lordotic curve with a normal promontory angle.

Vertebral bodies have normal height and signal intensity, without any sign of mass or fracture or compression.

The intervertebral disc spaces are preserved, with no sign of desiccation.

Also no sign of significant disc bulging or protrusion is seen.

The intervertebral neural foramina appear normal.

Visualized parts of cord, conus medullaris and also cauda equina appear normal in position, size and signal intensity, without any edema, hemorrhage or mass.

No extra or intra dural mass is seen.

Posterior elements appear normal.

Paravertebral soft tissue structures are unremarkable.

Bilateral mammogram:

Indication exam: A Y/O female with opportunistic screening

Comparison and correlation:

Previous mammogram is available for comparison dated

Previous study is not available for comparison.

first mammogram

Family history: negative positive for breast cancer in first and second degree relatives

Reported physical exam findings: normal breast exam.

Overall breast composition:

(ACR C) both breasts are heterogeneously dense, which may obscure small masses. In this type of breast ultrasound can be helpful.

(ACR A) the breasts are almost entirely fatty. Mammography is highly sensitive in this setting.

(ACR B) there are scattered areas of fibroglandular density.

(ACR C) some areas in the breast are sufficiently dense to obscure small masses. In this type of breast ultrasound can be helpful.

(ACR D) the breasts are extremely dense, which lowers the sensitivity of mammography. In this type of breast ultrasound can be helpful.

Findings:

No mass, malignant looking microcalcification or architectural distortion is seen in both breasts.

No skin thickening or nipple retraction is seen.

Assessment:

BI-RADS 0, further imaging needed

BI-RADS I, normal mammography

BI-RADS II, benign findings

BI-RADS III, probably benign findings

BIRADS IV, suspicious findings

Recommendation:

Routine screening mammography is recommended.

Breast and axillary sonography is recommended.

Correlation with diagnostic mammogram (FCMV on and true lateral) and targeted ultrasound is recommended for further evaluation.

Biopsy should be done in the absence of clinical contraindications.

Short term follow up mammography is recommended in six months.

Brain MRI with & without Contrast:

Multiplanar multisequential MR images through the brain with & without gadolinium injection were taken, which demonstrate:

Normal signal intensity of both cerebral hemispheres is noted with no evidence of mass, hemorrhage or acute ischemic infarction.

No hydrocephalus or shift of midline structures is seen.

Posterior fossa structures, including cerebellar hemispheres show normal signal intensity, without any sign of mass, hemorrhage or acute ischemic infarction.

There is n

o sign of increased intracranial pressure.

7th-8th nerve complexes appear normal.

Pituitary gland appears normal, with no sign of mass.

No extra-axial mass, hematoma or fluid collection is seen.

The paranasal sinuses and mastoid air cells show normal development and pneumatization.

The orbital contents are unremarkable.

No abnormal enhancing lesion is noted.

Impression: Normal Brain M.R.I

Brain MRI without Contrast:

Multiphase multisequential MR images through the brain without gadolinium injection were taken, which demonstrate:

Normal signal intensity of both cerebral hemispheres is noted with no evidence of mass, hemorrhage or acute ischemic infarction.

No hydrocephalus or shift of midline structures is seen.

Posterior fossa structures, including cerebellar hemispheres show normal signal intensity, without any sign of mass, hemorrhage or acute ischemic infarction.

There is no sign of increased intracranial pressure.

7th-8th nerve complexes appear normal.

Pituitary gland appears normal, with no sign of mass. No suprasellar mass lesion is noted

No extra-axial mass, hematoma or fluid collection is seen.

The paranasal sinuses and mastoid air cells show normal development and pneumatization.

The orbital contents are unremarkable.

Conclusion:

? Normal Brain M.R.I.

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No extra-axial mass, hematoma or fluid collection is seen.

The paranasal sinuses and mastoid air cells show normal development and pneumatization.

The orbital contents are unremarkable.

Conclusion:

? Normal Brain M.R.I.

MR ENTEROGRAPHY (MRI Abdomen & pelvic without & with contrast):

PROTOCOL: Pre contrast images were obtained with cine fluoro imaging as well as fat saturated T2 and also T1 images of small bowel. Pre contrast T1 W flash images were also obtained .Post contrast images were obtained using T1 flash images as well as T2 T1 and T1 images.

FINDINGS:

Jejunum folds appear normal without nodularity. No small bowel obstruction

trusion is seen . Ileal loops are also normal . No area of abnormal wall enhancement or thickening or obvious was noted. No evidence of fistula formation was noted. Comb sign was not noted. Other visualized abdominal viscera were also unremarkable. The liver is normally positioned and has normal size and smooth borders.

Its internal structure is normal with no focal abnormalities of signal intensity. The intrahepatic and extra hepatic bile ducts are not distended. The portahepatis appears normal.

The gallbladder displays a normal size, Smooth borders and homogenous contents.

The spleen is orthotopic and of normal size. It has smooth outer contours and a homogenous internal structure. The pancreas is normal in size and position.

The head and tail of the organ have smooth, lobulated outer contours and normal internal structures.

The pancreatic duct is unobstructed.

Both adrenal gland are normal in position and size and the adrenal crura are normally developed. The adrenal compartment is unremarkable.

Both kidneys are normal in size and position. The renal parenchyma shows normal width and structure.

The renal pelvis and calices are normal. The urinary drainage tract is unobstructed.

Major vessels and the para-aortic region appear normal, with no evidence of lymphadenopathy.

The pelvic inlet appears normal with normal configuration of the iliac wings and iliopsoas muscles.

No abnormalities are found in imaged bowel structures and there are no signs of wall thickening or mass lesions.

The distended urinary bladder appears normal and has normal wall thickening.

***The seminal vesicles are of normal size. The angle between the bladder and seminal vesicles is normal on each side.

***The prostate shows a normal size and configuration.

***The uterus is anteverted and has normal internal structure.

***The adnexa appear normal on both sides.

The vessels of the lesser pelvis are normal in their course and caliber. There is no apparent lymphadenopathy.

The femoral heads are normally shaped and articulate normally with the acetabula .

They have normal bone-marrow signal characteristics.

The soft tissues show no abnormalities.

M.R Angiography of the Brain:

Multiphase multisequential MR images through the brain without contrast were taken, which demonstrate:

The internal carotid arteries show normal course and caliber and are symmetrically disposed.

Each carotid siphon is normal, showing no displacement or extrinsic compression. Intraluminal signal intensity is homogenous.

Middle cerebral artery arises normally from the internal carotid on each side and forms normal insular loops. There is no circumscribed vascular narrowing and dilatation. The vessel lumen shows homogenous signal intensity.

Anterior cerebral artery shows no signs of narrowing or displacement.

The basilar artery shows a normal course and caliber and divides into normal-size posterior cerebral arteries.

Anterior and posterior communicating arteries on each side are normally developed and of normal size.

No segments show convolution or circumscribed dilatation.

Other evaluable portions of the neurocranium show no abnormalities.

Impression: Normal brain M.R. Angiography.

M.R. Angiography of the Cervical Arteries:

Multiphase multisequential MR images through the cervical arteries without contrast were taken, which demonstrate:

The aortic arch presents smooth walls and normal configuration. The brachiocephalic trunk arises normally and undergoes a normal division into the subclavian

artery, common carotid artery, and right vertebral artery.

The Lt. Common carotid artery arises directly from the aortic arch, has a normal caliber, and shows no luminal narrowing or filling defects. The vertebral artery appears normal.

The carotid bifurcation occurs at a normal level on each side and is normally shaped.

The external carotid artery and particularly the internal carotid artery are symmetrical on each side and have normal calibers.

There is no circumscribed narrowing or expansion.

The vessels display a homogenous intraluminal signal.

The carotid siphon appears normal, showing no displacement or extrinsic compression.

The vertebral arteries are symmetrically disposed and take a normal course.

They show normal luminal diameters with no filling defects or caliber irregularities as far as the basilar artery.

The portion of the neck than are images and Evaluable show no abnormalities.

Impression: Normal Cervical M.R. Angiography.

MRCP:

The study was done as usual protocol which demonstrates:

Gall bladder is well distended with no sign of SOL.

Intrahepatic bile ducts are not dilated.

Cystic duct is normal.

The CBD is normal in course with no dilatation or stone.

The visualized part of pancreatic duct is normal with no sign of dilatation or narrowing.

M.R.V. of the Brain:

M.R.I. of the venous cranial vessels demonstrates:

Superior sagittal sinus shows normal caliber with normal arrangement of draining superficial cerebral veins.

The great cerebral vein and inferior sagittal sinus appears normal.

The transverse sinus presents a normal caliber and shows slight asymmetry between the right and left side.

The other evaluable deep cerebral veins are normally developed and patent.

No venous segments contain filling defects or show loss of flow voids.

Other evaluable portions of the neurocranium show no abnormalities.

Conclusion:

? Normal M.R. Venography of Brain.

SPIRAL CT SCAN OF NECK(CORONAL, SAGITTAL, RECONSTRUCTION) WITH CONTRAST:

Multiple axial, coronal sagittal and reconstruction images through the neck demonstrate:

Normal deep soft tissue structures .

Parotid space as well as para pharyngeal spaces are clear without any lymphadenopathy, collection or mass.

Laryngeal structure such as vocal muscle and cartilage have normal configuration and showing no evidence of any infiltrative lesion or mass.

There is no evidence of definite mass in larynx.

Bony parts are unremarkable.

Submandibular and parotid glands and thyroid glands are normal.

SPIRAL CT SCAN OF NECK(CORONAL, SAGITTAL, RECONSTRUCTION) WITHOUT CONTRAST:

Multiple axial, coronal sagittal and reconstruction images through the neck demonstrate:

Normal deep soft tissue structures .

Parotid space as well as para pharyngeal spaces are clear without any lymphadenopathy, collection or mass.

Laryngeal structure such as vocal muscle and cartilage have normal configuration and showing no evidence of any infiltrative lesion or mass.

There is no evidence of definite mass in larynx.

Bony parts are unremarkable.

Submandibular and parotid glands and thyroid glands are normal.

SPIRAL CT SCAN OF ORBITS WITHOUT CONTRAST AXIAL AND CORONAL :

Multiple axial and coronal cuts through the orbits reveal:

Bilateral intact eye globe proper.

There is no evidence of preseptal or retro-orbital inflammatory process.

The optic nerves bilaterally are symmetrical and normal in caliber.

The para ocular musculature is normal.

The orbital foramen are bilaterally normal with no evidence of erosion.

No sign

of supra sellar mass lesion is seen.

Pelvic MRI with & without contrast :

The pelvic inlet appears normal with normal configuration of the iliac wings and iliopsoas muscles.

No gross abnormalities are found in images bowel structures, and there are no signs of wall thickening or mass lesions.

Urinary bladder appears normal and has a normal wall thickness.

Uterus has normal size and signal intensity with no sign of mass lesion.

Both adnexa are unremarkable.

There is no evidence of lymphadenopathy.

The femoral heads are normally shaped and articulate normally with the acetabula. They have normal bone marrow signal characteristics.

Soft tissues show no abnormalities.

Pelvic MRI with & without contrast :

The pelvic inlet appears normal with normal configuration of the iliac wings and iliopsoas muscles.

No gross abnormalities are found in images bowel structures, and there are no signs of wall thickening or mass lesions.

Urinary bladder appears normal and has a normal wall thickness.

Prostate and seminal vesicles are normal in signal intensity without mass lesion.

There is no evidence of lymphadenopathy.

The femoral heads are normally shaped and articulate normally with the acetabula. They have normal bone marrow signal characteristics.

Soft tissues show no abnormalities.

Dynamic MRI of the pituitary gland with & without contrast:

Multiplanar multisequential MR images through the pituitary gland were taken as usual protocol before and after contrast administration which demonstrate:

The size, position, and configuration of the sella are normal. The floor and walls of the sella are smooth and well-defined.

The pituitary is normal in size, shape, and position.

The pituitary tissue shows normal signal characteristics both before and after contrast injection, with no circumscribed abnormalities of signal intensity.

The infundibulum is centered and of normal size. The optic chiasma and suprasellar C.S.F space appear normal.

The cavernous sinus and imaged portions of the internal carotid artery and carotid siphon are unremarkable.

The sphenoid sinus is clear and pneumatized.

Dynamic sequence shows no time differential in the enhancement of different pituitary area.

Conclusion: Normal Pituitary gland MRI.

SPIRAL CT SCAN OF PARA NASAL SINUSES WITHOUT CONTRAST (axial and coronal):

Multiple axial and coronal images through the paranasal sinuses are obtained which reveals:

All para nasal sinuses are well aerated with no sign of sinusitis or mass lesion.

The osteomeatal complexes bilaterally are patent.

Nasal septal deviation is in mid line.

Nasal turbinates have normal shape.

Pterygopalatine fossa have normal symmetrical shape.

Maxillofacial structures show normal symmetrical anatomic configuration.

Bony orbits demonstrate normal symmetrical shape.

SPIRAL CTA OF PULMONARY VESSELS (CORONAL, SAGITTAL, 3D RECONSTRUCTION) PTE WITH injection CONTRAST:

Multiple axial, coronal sagittal and 3D reconstruction images through the chest and mediastinum with injection of contrast demonstrate:

No sign of PTE is seen.

Normal lung markings.

No active pulmonary infiltration seen.

There is no sign of interstitial lung disease.

The bronchial system are visualized to have normal caliber with no sign of peribronchial infiltration.

No abnormal dilatation of bronchial system or bronchiectasis is noted.

Heart size is normal.

No mediastinal or hilar lymphadenopathy is seen.

Bony thorax and soft tissue structures are apparently normal in anatomic configuration and density.

No s

ign of pleural effusion is seen.

RETROGRADE URETHROGRAPHY:

After insertion of Foley catheter into the distal part of penile urethra and injection of water soluble contrast agent the study reveals:

No evidence of stenosis, filling defect or mass effect seen in the course of urethral canal.

Free passage of contrast agent is demonstrated into the bladder.

M.R.I. of the right Knee Joint without Contrast:

Multiplanar multisequential MR images through the right knee joint, without gadolinium injection were taken which demonstrate:

There is no sign of significant joint effusion.

ACL and PCL are normal in size and signal intensity, without any sign of definite tear.

Medial and lateral menisci appear normal in signal intensity, without any sign of significant degeneration or tear.

Medial and lateral collateral ligaments are intact without any definite tear.

The quadriceps and patellar tendons are normal in size and signal intensity, without any evidence of tear.

The articular cartilage is preserved with no sign of degenerative changes or cartilaginous loss.

The patellofemoral joint is preserved without any sign of dislocation.

Visualized bony structures show normal signal intensity and configuration without any sign of bone bruise, fracture or mass.

Surrounding soft tissue structures are unremarkable without mass lesion or fluid collection.

Impression: Normal Knee Joint M.R.I

CORONARY CT CALCIUM SCORING REPORT:

Quality of scan: Good.

Agatston calcium score: 0

MRI of shoulder without contrast:

Multiplanar multisequential MR images through the shoulder without contrast administration were taken which demonstrate:

No effusion is seen in shoulder joint.

Rotator cuff tendons have normal signal intensity with no sign of tear.

Visualized bony parts including humerus show no sign of fracture or mass lesion.

The humeral head has normal configuration and articulates properly with the normally developed glenoid. The articular surfaces are smooth and joint spaces are preserved.

The glenoid labrum is intact without sign of tear.

The acromioclavicular joint has normal configuration, with no hypertrophy.

Long head of biceps demonstrate normal signal intensity and thickness and is well located in bicipital groove.

Deltoid muscles as well as other shoulder girdle muscle have normal signal intensity and anatomic distribution without sign of mass.

M.R.I OF THE SACROILIAC JOINT without contrast:

The sacroiliac joints are normally shaped with normal development of the sacrum and iliac wings and a normal ? appearing lumbosacral junction.

The joint space is of normal width on both sides.

The joint contours are smooth and sharply defined.

The subchondral bone marrow appears normal.

The are no marginal osteophytes.

The sacrum and iliac wing also contain normal bone marrow and present smooth, intact cortical boundaries.

The sacral neuroforamina are of normal width.

The nerve filaments show a normal course and diameter, and the width of the sacral spinal canal is normal.

The muscles and the imaged organs of the lesser pelvis show no abnormalities.

Impression: Normal sacroiliac joints .

BARIUM SWALLOW:

There was normal swallowing function fluoroscopically.

No evidence of constant narrowing or dilatation in the course of the esophagus.

No evidence of filling defect or ulceration.

SPIRAL CT SCAN OF TEMPORAL BONE AND POSTERIOR FOSSA AXIAL AND CORONAL WITHOUT CONTRAST:

Multiple axial and coronal images through the posterior fossa and temporal bone demonstrate:

The inte

rnal auditory canals bilaterally are visualized to be symmetrical.

The tympanic cavity are clear of any soft tissue infiltration.

The ossicular chain are visualized to be normal in density and position .

The mastoid air cells bilaterally are well aerated.

The external auditory canals do not show any soft tissue within the lumen.

The visualized bony structure in skull base are normal.

M.R.I OF THE Lt. THIGH with & without contrast:

Coronal axial & sagittal T1WI and axial and coronal T2WI before & after contrast administration demonstrated:

Normal femoral bone with normal thickness of the cortex and signal intensity of bone marrow.

All imaged muscles demonstrated normal configuration, internal signal intensity & interrelationships. Neurovascular bundles are normal.

Subcutaneous fat demonstrated normal thickness and internal signal intensity.

After injection of contrast, no any enhancing lesion is detected on any imaged segment.

Conclusion: normal thigh M.R.I.

Thoracic spine MRI without contrast:

Multiplanar multisequential MR images through the thoracic spine, without gadolinium injection were taken which demonstrate:

The thoracic spine shows a normal degree of kyphosis.

All visualized vertebral bodies have normal size and signal intensity, without any sign of mass or fracture or compression.

The intervertebral disc spaces are preserved, with no sign of desiccation. Also no sign of significant disc bulging or protrusion is seen.

The intervertebral neural foramina appear normal.

Visualized parts of cord appear normal in size and signal intensity, without any sign of edema, hemorrhage or mass.

The paravertebral soft tissues show no abnormalities.

SPIRAL CT SCAN OF ABDOMEN AND PELVIC +DYNAMIC OF

LIVER WITH CONTRAST + CORONAL AND SAGITTAL RECONSTRUCTION:

Multiple axial, coronal and sagittal reconstruction images through the abdomen and pelvic after IV and oral contrast in triphasic (arterial, venous & delayed images) administration demonstrate:

The liver is normal in size with normal parenchymal density with no evidence of mass or biliary dilatation.

The G.B is unremarkable.

Pancreas has normal size and density with no sign of S.O.L.

Spleen has normal size and density with no sign of S.O.L.

Both adrenal glands are normal.

Both kidneys are normal in size and density with no sign of stone, stasis or abnormal density.

No hydronephrosis or ureteral dilatation is seen.

There is no sign of peripancreatic or para aortic lymphadenopathy.

There is no sign of mesenteric stranding. The hollow viscous structures have normal distribution.

Urinary bladder is normal with no sign of stone or wall thickening.

Other genito-urinary structures in pelvis show normal density and anatomic configuration.

There is no sign of pelvic side wall lymphadenopathy.

The visualized part of the bony skeleton are normal in density and shape.

No free fluid is noted in abdominopelvic cavity.

SPIRAL CT SCAN THORACIC SPINE WITHOUT CONTRAST WITH SAGITTAL CORONAL 3D RECONSTRUCTION :

The visualized vertebrae have normal height and alignment .

The vertebral bodies have normal shape with no evidence of fracture.

The posterior elements also are normal with no evidence of fracture line or dislocation.

No lytic or blastic bony lesion is noted.

UGI:

There is normal transit through the jejunum and ileum.

Individual loops of small intestine show no abnormalities.

The terminal ileum is properly shaped and terminates normally in the lower pole of the cecum.

Interpretation: The UGI series appea

r normal.

VCUG:

Scout film shows no evidence of radiopaque density.

The gas pattern is normal.

Bony structures are unremarkable.

After insertion of foley catheter and instillation of water soluble contrast agent , free filling of U.B is seen.

After D/C of foley and start of urination study reveals:

There is no sign of V.U reflux.

Urethra is normal too.

M.R.I OF THE WRIST without contrast:

The bone comprising the wrist present a normal configuration.

The radial joint angle is normal.

The carpal bones show normal shape and relationship to one another and to the radiocarpal and carpometacarpal joints.

The articular surfaces are smooth and congruent with normal cortical thickness and normal width of the joint spaces.

There are no osteophytes and no subchondral signal changes.

The bone marrow signal is normal.

The ulnar (triangular) disc exhibits normal configuration and normal signal characteristics.

The intraosseous ligaments also appear normal.

The carpal tunnel is of normal width and transmits tendons that are normal in width and position.

The median and ulnar nerves appear normal.

The metacarpals and phalanges have normal margins and normal bone marrow signal intensity.

The soft tissues are normal.

Impression: Normal WRIST & HAND.